

AIDS-Associated Kaposi Sarcoma

EPIDEMIOLOGY

A major harbinger of the AIDS pandemic was the emergence of clusters of young homosexual men suffering from Kaposi sarcoma (KS) in large U.S. cities in 1981. During the first decade of the AIDS pandemic, KS was diagnosed as the AIDS-defining illness in more than 20 percent of HIV-1-infected patients in Europe. Despite a decline in recent years, KS remains the most frequent AIDS-associated tumor in men who have sex with men. Other patient groups at risk for AIDS—including intravenous drug users, hemophiliacs, blood transfusion recipients, and children born to HIV-positive mothers in industrialized countries—are far less commonly affected.

The situation differs in Africa, where KS is the most common tumor among HIV-1-infected individuals across all risk groups, including children with AIDS.

CLINICAL FEATURES

AIDS-associated KS differs from classic KS by its more rapid progression and multifocal dissemination. Early lesions appear as small, oval, violaceous macules that quickly evolve into plaques and nodules. These lesions are often present at multiple sites at disease onset and tend to progress rapidly.

In contrast to other forms of KS, initial lesions in AIDS patients frequently develop on the face—particularly the nose, eyelids, and ears—and on the trunk, where they often align along the skin's relaxed tension lines (Fig. 128-3). If left untreated, disseminated lesions may coalesce into large plaques affecting extensive areas of the face, trunk, or extremities, potentially causing functional impairments.

Oral mucosal involvement is common and is the presenting site in 10% to 15% of AIDS-related KS cases (Fig. 128-4). Pharyngeal involvement may also occur, leading to difficulties in eating, speaking, and breathing.

EXTRACUTANEOUS INVOLVEMENT

Extra-cutaneous involvement occurs more rapidly and severely in AIDS-associated KS compared to classic KS. In addition to the oral mucosa, KS lesions are most frequently found in lymph nodes, the gastrointestinal tract, and the lungs.

Although gastrointestinal KS typically occurs alongside cutaneous disease, isolated gastrointestinal involvement can occur. The stomach and duodenum are common sites, and lesions in these areas may cause symptoms such as gastrointestinal bleeding or obstruction. These submucosal lesions are often visible during gastroscopy but may be missed on biopsy due to their depth.

Pulmonary KS can lead to respiratory symptoms including bronchospasm, cough, and progressive respiratory insufficiency. Diagnosis is best achieved via bronchoscopy with transbronchial biopsy.

DIFFERENTIAL DIAGNOSIS

Differential diagnoses for cutaneous KS vary depending on the clinical stage and are outlined in Box 128-1.

HISTOPATHOLOGY

The histopathologic features of KS depend on the stage of lesion development. Early patch-stage lesions show subtle changes, primarily an increase in dermal vasculature lined by slightly atypical endothelial cells (Fig. 128-5).